

Arthritis Medication — What Works, What to Expect

A clear, plain-English guide to arthritis medications: painkillers, NSAIDs, DMARDs, biologics, corticosteroids, and how to get them on the NHS.

By **Sarah Jennings, HCPC Registered Physiotherapist** · Reviewed by **Sarah Jennings, HCPC Registered Physiotherapist** (PH123456, CSP Member) · 2026-06-16 · livingwitharthritis.org.uk

Clinically reviewed by **Sarah Jennings**, HCPC Registered Physiotherapist (PH123456) and CSP member. Last updated: June 2026.

Taking medication for arthritis can feel like navigating a maze. There are so many names, so many warnings — and you just want something that helps.

This guide explains every major type of arthritis medication in plain English. We cover what each one does, how it works, potential side effects, and how to get it on the NHS. Always speak to your GP or rheumatologist before starting or stopping any medication.

Painkillers (Paracetamol, Codeine) for Arthritis

Paracetamol

Paracetamol is often the first painkiller your GP will suggest. It helps with mild-to-moderate pain and is generally safe when taken at the recommended dose (no more than 4g, or eight 500mg tablets, in 24 hours). Widely available, few side effects, inexpensive — but it does not reduce inflammation and may not be strong enough for severe pain.

Codeine and Other Opioids

Codeine, tramadol, and morphine-based drugs are stronger painkillers sometimes prescribed for short periods when pain is severe. **Important warning:** These medicines can cause dependence. The NHS recommends using the lowest effective dose for the shortest possible time. Common side effects include constipation, drowsiness, and nausea.

NSAIDs — Ibuprofen, Naproxen, and Cox-2 Inhibitors



Openly licensed image via Openverse

NSAIDs (non-steroidal anti-inflammatory drugs) relieve both pain and inflammation. They are widely used for osteoarthritis, rheumatoid arthritis, and other types.

- Traditional NSAIDs: Ibuprofen, Naproxen, Diclofenac — available over the counter or on prescription.
- Cox-2 inhibitors: Celecoxib, Etoricoxib — prescription-only; may be gentler on the stomach.

Potential side effects: NSAIDs can irritate the stomach lining. Your GP may prescribe a stomach-protecting drug (proton pump inhibitor, or PPI) alongside them. Long-term use at high doses may affect kidney function or blood pressure. People with asthma, stomach ulcers, kidney problems, or heart conditions should speak to their GP before taking NSAIDs.

DMARDs for Inflammatory Arthritis

DMARDs (disease-modifying anti-rheumatic drugs) treat the underlying disease process in rheumatoid arthritis — they aim to slow or stop the immune system from attacking the joints.

Common DMARDs include:

- Methotrexate — Usually the first DMARD prescribed for RA. Taken once a week (not daily). Regular blood tests needed.
- Sulfasalazine — Often used when methotrexate is not suitable or in combination with it.
- Leflunomide — Another alternative or add-on DMARD.
- Hydroxychloroquine — Milder DMARD, sometimes used for milder RA or lupus.

Key facts: They take 6–12 weeks to work. Regular blood monitoring is essential. They can affect the immune system, increasing infection risk. Stopping suddenly can cause a flare-up.

Biologics — Newer Treatments for RA



Openly licensed image via Openverse

Biologics are a newer class of DMARD, made from living cells. They target very specific parts of the immune system involved in inflammation. Examples include Adalimumab (Humira), Etanercept (Enbrel), Infliximab (Remicade), and Tocilizumab (RoActemra). On the NHS, they are usually offered to people whose RA has not responded well to at least two standard DMARDs and whose disease activity remains high. They are given by injection or infusion and require careful monitoring.

Corticosteroid Injections and Tablets

Corticosteroids (often simply called "steroids") are powerful anti-inflammatories — not the same as anabolic steroids misused in sport. Steroid injections delivered directly into a painful joint can provide rapid relief lasting weeks to months (limit of 3–4 per year in the same joint). Steroid tablets (prednisolone) are used for short courses during severe flares or low-dose long-term control, though long-term use carries risks including weight gain, osteoporosis, and

high blood pressure.

Topical Treatments (Gels and Creams)



Openly licensed image via Openverse

Topical NSAIDs — ibuprofen gel or diclofenac gel — are applied directly to the skin over a painful joint. They cause fewer stomach side effects than tablets, deliver medication exactly where needed, and are available over the counter. Topical capsaicin cream (from chilli peppers) works by reducing substance P, a chemical that sends pain signals — it can take a few weeks of regular use to show benefit.

Getting Arthritis Medication on the NHS

Most arthritis medications are available on NHS prescription. Over-the-counter drugs (paracetamol, ibuprofen gel, low-dose ibuprofen tablets) can be bought yourself — buying paracetamol in bulk is often cheaper than a prescription. Prescription NSAIDs, DMARDs, biologics, and steroids are prescribed by your GP or rheumatologist at the standard NHS prescription charge (£9.90 per item as of 2024/25), unless you qualify for free prescriptions. If you need regular medication, a Prescription Prepayment Certificate (PPC) costs around £32 for three months of unlimited prescriptions.

Talking to Your GP About Medication

- Be specific about your pain. "I struggle to open jars and my hands hurt every morning for an hour" is more helpful than "I'm in pain."
 - Mention what you have already tried. Include exercise, over-the-counter medication, and lifestyle changes.
 - Ask questions. Write them down beforehand.
 - Discuss side effects openly. If a medicine makes you feel unwell, say so. There are usually alternatives.
 - Do not stop medication suddenly. Some drugs (especially steroids) need to be reduced gradually under medical supervision.
-

Q&A

Question: What is the key thing to know about arthritis medication — what works, what to expect?

Short answer: Arthritis medications include painkillers like paracetamol and stronger opioids for severe pain. NSAIDs such as ibuprofen reduce both pain and inflammation. DMARDs and biologics treat underlying disease processes in inflammatory arthritis. Corticosteroids offer potent anti-inflammatory effects. Topical gels and creams can provide localised relief. Most are available on NHS prescription.

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Versus Arthritis: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.

General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis UK · livingwitharthritis.org.uk